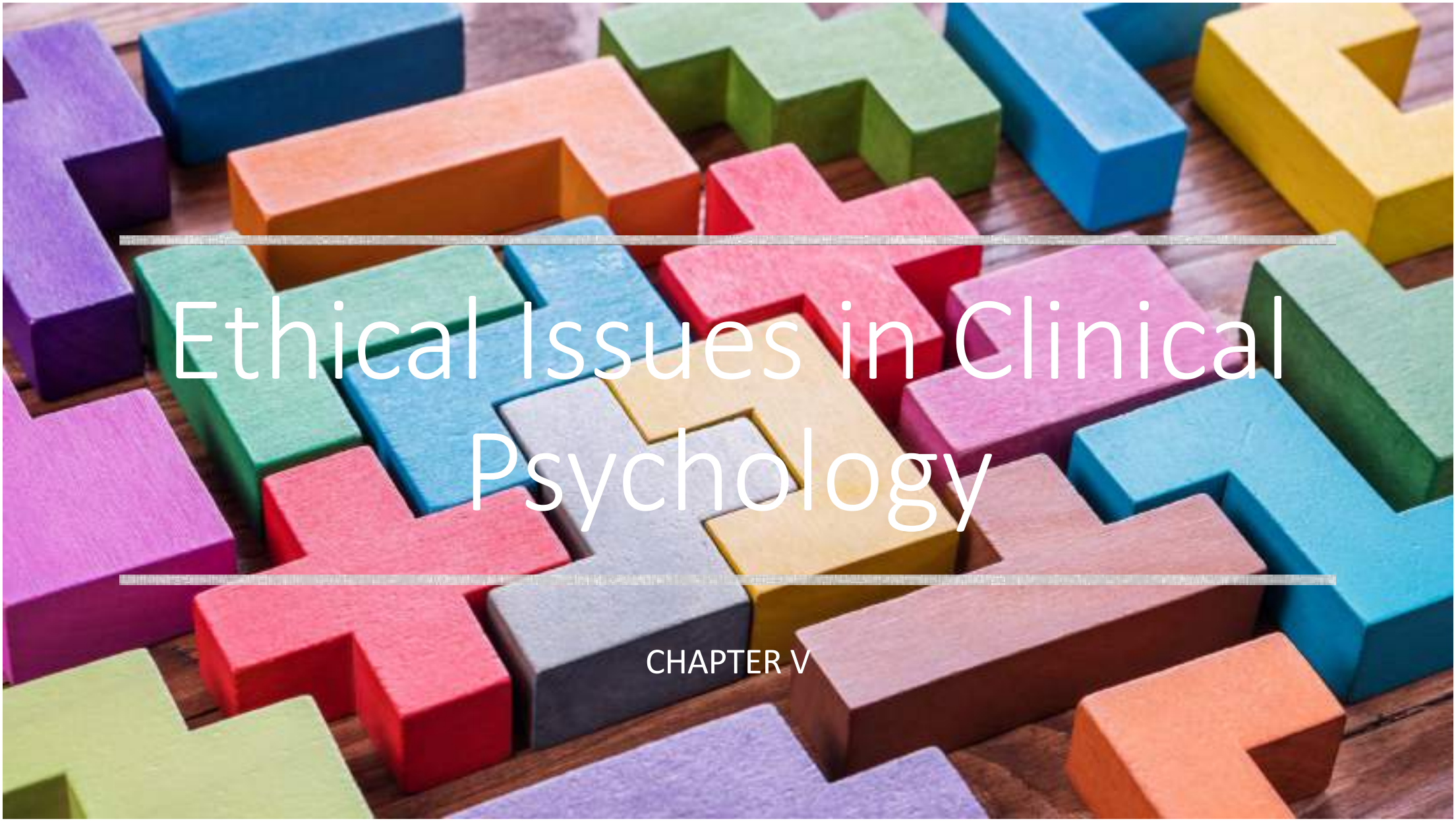


Ethical Issues in Clinical Psychology

CHAPTER V

APA Code of Ethics

- The current American Psychological Association (2002) ethical code features two distinct sections: **General Principles** and **Ethical Standards**
- The items in the **General Principles** section are aspirational. In other words, they describe an ***ideal level of ethical functioning*** or how psychologists should strive to conduct themselves. They don't include specific definitions of ethical violations; instead, ***they offer more broad descriptions of exemplary ethical behavior***.

PAP Code of Ethics

The Psychological Association of the Philippines (PAP) adopted a Code of Ethics for Clinical Psychologists in the 1980s, and this Code has remained unchanged since then.

In 2007, the Board of Directors of the PAP resolved to constitute a committee to revise the Code of Ethics, with the goal of updating the Code and making it more inclusive and applicable to all psychologists.

PAP Ethical Principles

- Respect for the Dignity of Persons and Peoples
- Competent Caring for the Well-Being of Persons and Peoples
- Integrity
- Professional and Scientific Responsibilities to Society

Code of Ethics

In contrast to the General Principles section, the Ethical Standards section of the ethical code includes enforceable rules of conduct.

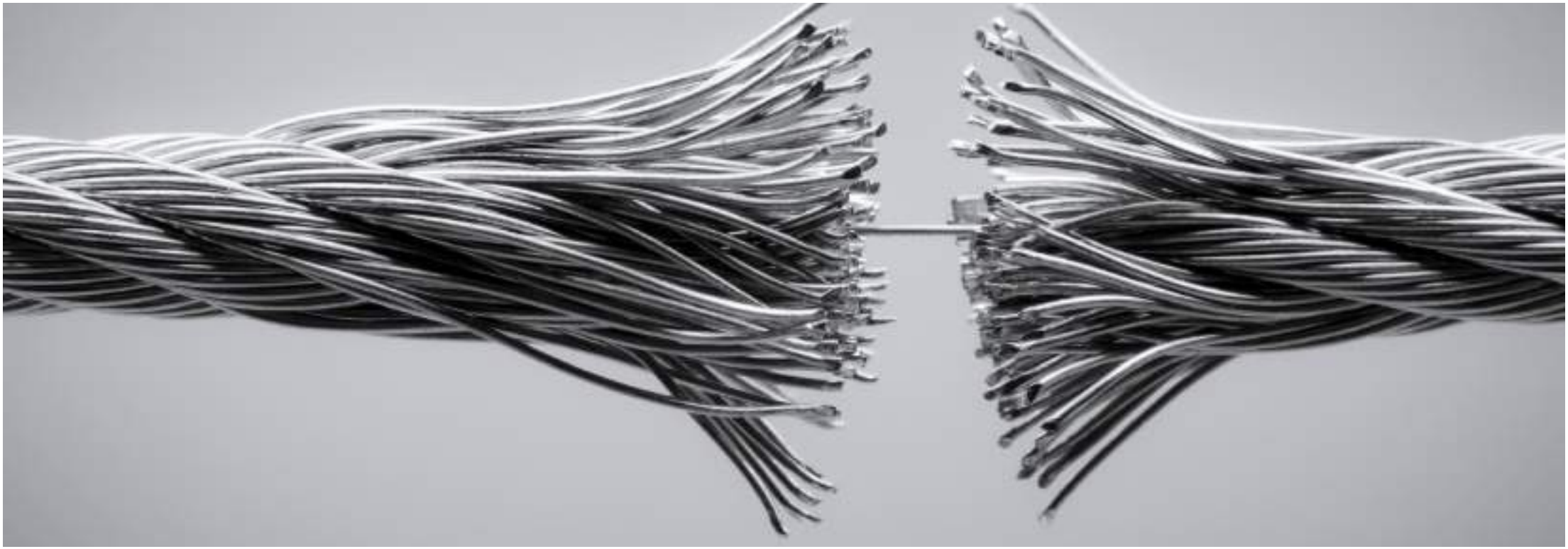
Thus, if a psychologist is found guilty of an ethical violation, it is a standard (not a principle) that has been violated. These standards are written broadly enough to cover the great range of activities in which psychologists engage, but they are nonetheless more specific than the general principles.

Although each general principle could apply to almost any task a psychologist performs, each ethical standard typically applies to a more targeted aspect of professional activity. The ethical standards are divided into 10 categories, and, collectively, these 10 categories include 89 individual standards.

Ethical Decision Making

Before turning to any step-by-step ethical decision-making model, psychologists best prepare themselves to deal with ethical dilemmas *by becoming generally ethical people with sound values*.

In other words, functioning as an ethical psychologist should not be a rote exercise in which a predetermined model for decision making is the sole guide. While such models can certainly enhance the psychologist's chances of making the most ethical decision, ***they work best when utilized by a person who has already examined his or her own values and aligned them with the ethics of the profession***



Confidentiality

Confidentiality

- There is good reason for the emphasis on confidentiality in the profession of psychology: *Our profession is entrusted by the public to provide professional services without sharing the private, personal details offered in the process.*
- However, the public may be unaware of the fact that **confidentiality is not absolute**. Although most people outside of the mental health profession may assume that psychologists hold all information confidential, the truth is that situations arise in which psychologists are obligated to break confidentiality.

Tarasoff and the Duty to Warn

In 1969, Prosenjit Poddar was a student at the University of California at Berkeley. He became romantically interested in Tatiana Tarasoff, and when their relationship did not advance as he hoped, his mental state worsened and he sought psychotherapy at the university counseling center from a psychologist, Dr. Lawrence Moore. During a session in August 1969, Poddar told Dr. Moore that he intended to kill Tarasoff. Dr. Moore believed that Poddar's comment was credible, so he broke therapist–client confidentiality and contacted campus police. The campus police interviewed Poddar but did not hold him, because he promised to avoid Tarasoff and seemed rational at the time of the interview. Poddar never returned to therapy. Two months later, on October 27, 1969, Poddar killed Tarasoff by stabbing and shooting her. Tarasoff's parents later sued Dr. Moore and the others involved in the case for wrongful death. The court found that the psychologist was liable for failure to warn Tarasoff of the danger.

Two years later, the court reheard the case and slightly revised their verdict, from a duty to warn to a duty to protect (which could involve something other than a direct warning by the therapist); however, the "duty to warn" language is still commonly used. Both "duty to warn" or "duty to protect" decisions suggest that the court saw Dr. Moore's actions as insufficient. Although he broke confidentiality and contacted campus police, the court determined that he should have made additional efforts to ensure that Tarasoff knew that she was in harm's way.

Tarasoff and the Duty to Warn

From the Tarasoff case, the duty to warn (and duty to protect) was born. **Since the Tarasoff case set the legal precedent, clinical psychologists (and other therapists) have understood that there are limits to their confidentiality agreements with clients and that they have a duty to warn or protect people toward whom their clients make credible, serious threats.**

As stated in the first Tarasoff ruling, "The confidential character of patient-psychotherapist communications must yield to the extent to which disclosure is essential to avert danger to others. **The protective privilege ends where the public peril begins**"

Certainly, the rationale behind the duty to warn is clear: In the Tarasoff situation, a young woman's life could have been spared if a warning had been issued.

When the Client Is a Child or Adolescent

- One challenge centers on the fact that for many children and adolescents, the establishment of a close, ***trusting relationship with a clinical psychologist depends on the extent to which the psychologist reveals details of one-on-one conversations with the child's parents***
- Simply put, ***kids might choose to withhold rather than discuss important personal issues if they know that their psychologists will subsequently share the information with the kids' parents***.
- Of course, as legal guardians, *parents are ultimately entitled to be informed about their children's progress in therapy; moreover, it's effective clinical practice to keep parents actively engaged in a child's therapy*.

When the Client Is a Child or Adolescent

Sometimes, clinical psychologists can ***discuss this confidentiality dilemma openly with child clients and their parents, and a mutually agreeable arrangement can be reached***

In other words, "therapists working with minor clients often negotiate an arrangement with their clients and their parents *whereby the parents will not expect to be informed of what the client discloses in therapy* ***except under specified conditions***"

Those specified conditions should, at the very least, include any situation in which ***children might be harmed by the behavior of others or themselves***, as well as ***general information*** about the child's psychological condition or progress in therapy.

Parents may still refuse…

"Psychotherapy works best when people have confidence in the privacy of their conversations. At the same time, parents do want to feel confident about their child's well-being and safety. Since parents were once teenagers, you certainly know that an adolescent may want to use therapy to talk about sex, alcohol, smoking, or other activities that parents may not approve of. Let's talk about how we can assure your child of confidentiality so s/he can talk openly about what's on her/his mind and at the same time assure your parents about your safety."

When the Client Is a Child or Adolescent

When parents do accept such an arrangement, they place quite a bit of trust in the psychologist's judgment. Consider some of the behaviors in which minor clients might engage:

smoking, drinking, sex, drug use, crime, and "cutting" (self-mutilation), to name a few.

At what point or under what conditions do these behaviors constitute harm that merits the psychologist's informing the child's parents? To what extent is the child's age a factor?

Example…

Consider Danica, a 17-year-old girl seeing Dr. Terry, a clinical psychologist. Danica's parents believe that Danica deserves some confidentiality with Dr. Terry, and they agree that Dr. Terry need not repeat the full contents of their sessions; however, they understandably insist that they be informed of any harm or danger that Danica may experience.

As the sessions progress, the therapeutic relationship strengthens, and Danica begins to reveal to Dr. Terry details of her life about which Danica's parents are unaware.

These details include the fact that Danica drinks alcohol about once a week (but does not get drunk), that she intentionally cut her forearm with a razor blade once a few months ago, and that one night she was a passenger in a car driven by a friend who may have been high.

Do any of these behaviors or situations call for Dr. Terry to inform Danica's parents? If not, how much would the behaviors have to intensify before they did? What consequences might Dr. Terry be able to expect if she did or did not tell Danica's parents? Would the answers differ if Danica was 14, or 11, or 8 years old?

Child abuse

- Every state has laws requiring mental health professionals to break confidentiality to report known or suspected child abuse
- The rationale behind such laws is like the rationale behind the Tarasoff ruling; namely, ***some situations demand that the clinical psychologist's primary responsibility shift to the immediate prevention of harm***.
- And, like duty-to-warn situations, child abuse situations often require the clinical psychologist to make difficult judgment calls. ***It can be exceedingly challenging to determine with confidence whether child abuse is likely to have taken place, especially with children who may not be entirely forthcoming, who may exaggerate claims against their parents, or whose communication skills may be limited***.
- The clinical psychologist's overarching goal of treatment may remain simple— the well-being of the child—but in cases in which child abuse is suspected, determining the means to attain this goal can become complex.

Child abuse

- As a final note on confidentiality, it is important to note the difference between legal standards and ethical standards. Although they may differ in rare situations
- Duty to warn in Tarasoff-like situations and required reporting of suspected **child abuse—often represent both legal standards and ethical standards.**
- That is, **state law typically requires such behavior by clinical psychologists, and the ethical code includes standards that are consistent with these laws**

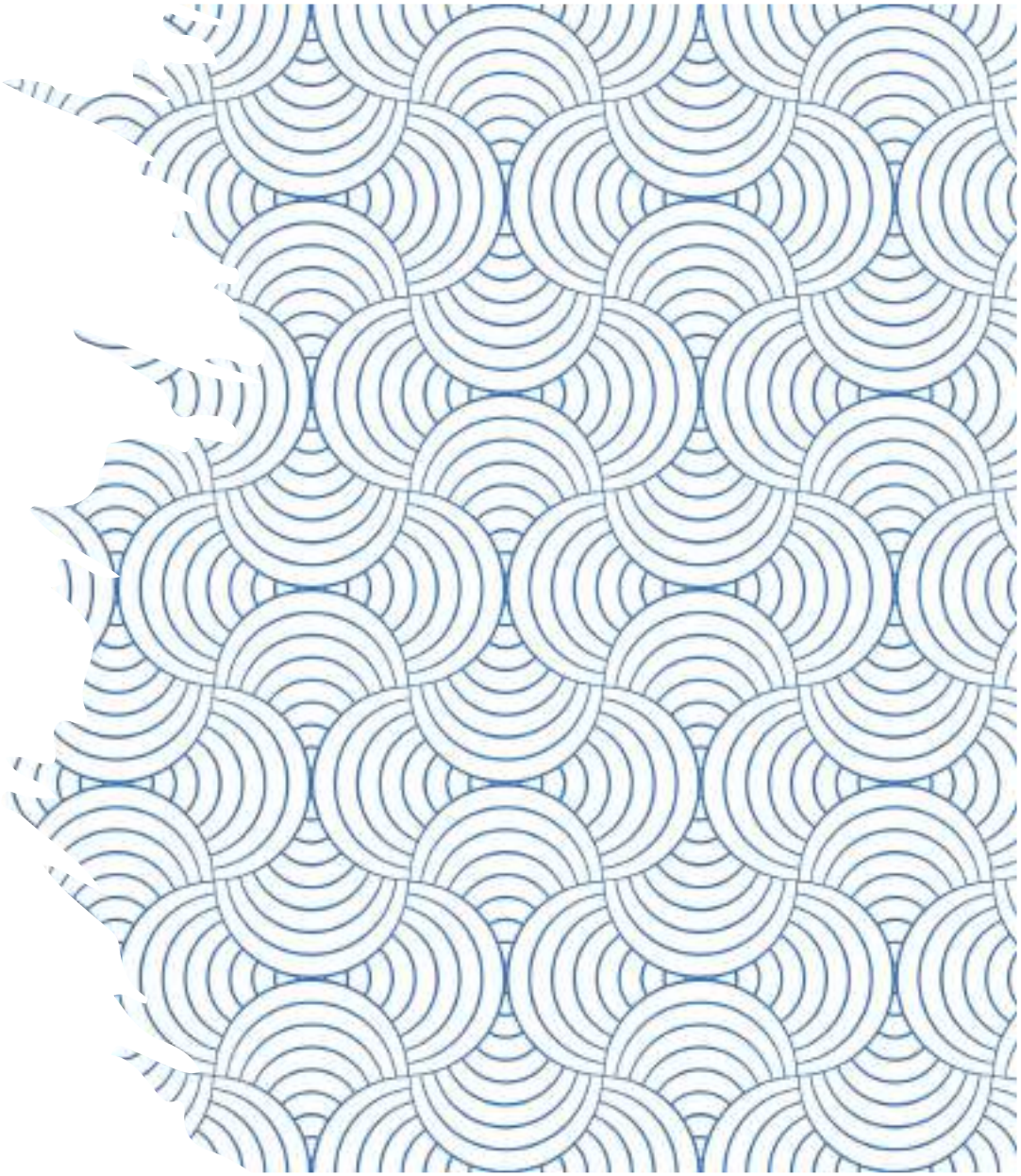
Exceptions to Confidentiality and Privileged Communication

- In determining when to breach confidentiality, therapists must consider the requirements of the law, the institution in which they work, and the clientele they serve. Because these circumstances are frequently not clearly defined by accepted ethics codes, counselors must exercise professional judgment.
- Whenever counselors are not clear about their obligations regarding confidentiality or privileged communication, it is critical to seek consultation and to document these discussions.
- All mental health practitioners and interns need to be aware of their duty to report in these situations and to know the limitations of confidentiality. Here are some other circumstances in which information must legally be reported by counselors:
    - When the therapist believes a client under the age of 16 is the victim of incest, rape, child abuse, or some other crime
    - When the therapist determines that the client needs hospitalization
    - When information is made an issue in a court action
    - When clients request that their records be released to them or to a third party

Informed Consent

- Research is certainly an important application of the ethical standards involving informed consent, but it is not the only one. Assessment and therapy also require informed consent according to the ethical code
- **It ensures the person with whom the psychologist is working the opportunity to become knowledgeable about the activities in which they may participate, and it facilitates an educated decision**. Moreover, it affords individuals the opportunity to refuse to consent if they so choose.
- Psychologists should offer information about the nature and purpose of the assessment; any relevant fees; the involvement of other parties, if any; and limits of confidentiality (e.g., duty-to-warn or child abuse situations).
- Psychotherapy requires an informed consent process as well. Ethical Standard 10.01 ("Informed Consent to Therapy") **explains that psychologists inform clients/patients as early as is feasible in the therapeutic relationship about the nature and anticipated course of therapy, fees, involvement of third parties, and limits of confidentiality and provide sufficient opportunity for the client/patient to ask questions and receive answers.**

Boundaries & Multiple Relationships



Boundaries and Multiple Relationships

- In general, it can be problematic for clinical psychologists to know someone professionally—as, say, a therapy client or student—and also to know that person in another way—as, say, a friend, business partner, or romantic partner.
- The term used to describe such situations is multiple relationships (although the term dual relationships has also been used).

Multiple Relationships occurs when a psychologist is in a professional role with a person and…

1. at the same time is in another role with the same person,
2. at the same time is in a relationship with a person closely associated with or related to the person with whom the psychologist has the professional relationship, or
3. promises to enter into another relationship in the future with the person or a person closely associated with or related to the person.

PAP Code of Ethics 2022

1. We shall refrain from entering into a multiple relationship if the multiple relationship could reasonably be expected to impair our objectivity, competence, or effectiveness in performing our functions as psychologists or psychometricians, or otherwise risks exploitation or harm to the person with whom our professional relationship exists.

If however, we shall find that, due to unforeseen factors, a potentially harmful multiple relationship has arisen, we shall take reasonable steps to resolve it with regard for the best interests of the affected person and maximum compliance with the Code of Ethics.

2. When we shall be required by law, institutional policy, or extraordinary circumstances to serve in more than one role in judicial or administrative proceedings, at the outset we shall endeavor to inform the authorities about our Code of Ethics, to clarify role expectations and extent of confidentiality and thereafter as changes occur. (See also Standards III.D, Avoiding Harm, and III.G, Third-Party Requests for Services.)

Example…

- Monique, a 36-year-old woman, is the therapy client of Dr. Davis, a clinical psychologist, Dr. Davis would be forming a multiple relationship if she became Monique's friend, business partner, or romantic partner. Additionally, Dr. Davis would be forming a multiple relationship if she became similarly involved with Monique's romantic partner, sibling, or close friend.
- For clinical psychologists considering this ethical standard, defining "closely associated or related" can be a challenge, especially considering the "six degrees of separation" nature of our cities and communities.

Defining Multiple Relationships

- Perhaps the most blatant and damaging are ***sexual multiple relationships***, in which the clinical psychologist becomes a sexual partner of the client. "Psychologists do not engage in sexual intimacies with current therapy clients/patients" (Standard 10.05, p. 1073).
- Such behavior represents a ***fundamental breach of the healthy therapist/client relationship and often results in significant psychological or emotional damage for the client.***
- The important issue is ***how psychologists deal with such feelings***: preferably by discussing them in consultation with other professionals or perhaps their own therapist, but certainly not by acting on them
- The nonprofessional involvement between client and clinical psychologist **need not be sexual to constitute a multiple relationship or to cause the client harm**.
- Psychologists may have opportunities to engage in a wide variety of nonsexual multiple relationships: **friendships, business or financial relationships, coworker or supervisory relationships, affiliations through religious activities, and many others**

What makes multiple relationships unethical?

- A psychologist refrains from entering into a multiple relationship if the multiple relationship could reasonably be expected **to impair the psychologist's objectivity, competence, or effectiveness in performing his or her functions as a psychologist, or otherwise risks exploitation or harm to the person with whom the professional relationship exists**.
- There are essentially two criteria for impropriety in a multiple relationship.
  - The first **involves impairment in the psychologist**; if the dual role with the client makes it difficult for the psychologist to remain objective, competent, or effective, then it should be avoided.
  - The second involves **exploitation or harm to the client**. Psychologists must always remember that the **therapist–client relationship is characterized by unequal power**, such that the therapist's role involves more authority and the client's role involves more vulnerability, especially because of some clients' presenting problems.
- Ethical psychologist must remain vigilant about exploiting or harming clients by clouding or crossing the boundary between professional and nonprofessional relationships. **Above all, the client's well-being, not the psychologist's own needs, must remain the overriding concern.**

What makes multiple relationships unethical?

It is possible to engage in a multiple relationship that is neither impairing to the psychologist nor exploitive or harmful to the client. (And in some settings, such as small communities, such multiple relationships may be difficult to avoid.)

However, multiple relationships can be ethically treacherous territory, and clinical psychologists owe it to their clients and themselves to ponder such relationships with caution and foresight.

Sometimes, major violations of the ethical standard of multiple relationships are preceded by ***"a slow process of boundary erosion***"—that is, a clinical psychologist may engage in some seemingly harmless, innocuous behavior that doesn't exactly fall within the professional relationship—labeled by some as a "boundary crossing"—and ***although this behavior is not itself grossly unethical, it can set the stage for future behavior that is.***

These harmful behaviors are often called "boundary violations" and can cause serious harm to clients, regardless of their initial intentions

III. Human Relations

PAP Code of Ethics 2022

Unfair Discrimination

In our work-related activities, we shall not discriminate against persons based on age, gender, gender identity, race, ethnicity, culture, national origin, regional identity, religion, sexual orientation, exceptionality, occupation, socioeconomic status, educational background, or any basis proscribed by law.

Sexual harassment

We do not engage in sexual harassment as defined in the Philippine Anti-Sexual Harassment Act (RA No. 7877).

G. Sexual Relationships with Students and Supervisees

We must not engage in sexual relationships with students or supervisees who are in our department, agency, or training center or over whom we have or are likely to have evaluative authority. (See also Standard III.E, Multiple Relationships.)

Other Harassment

We shall not knowingly harass or demean persons with whom we interact in our work on the bases of those persons' age, sex, gender identity, race, ethnicity, culture, national origin, regional identity, religion, sexual orientation, exceptionality, language, occupation, or socioeconomic status.

Avoiding Harm

We shall take reasonable steps to avoid harming our clients/patients, students, supervisees, research participants, organizational clients, and others with whom we work, and to minimize harm where it is foreseeable but unavoidable.

Slippery Slope

Consider Dr. Greene, a clinical psychologist in private practice. Dr. Greene finishes a therapy session with Annie, a 20-year-old college student, and soon after the session, Dr. Greene walks to his car in the parking lot. On the way, he sees Annie unsuccessfully trying to start her car. He offers her a ride to class, and she accepts. As they drive and chat, Annie realizes that she left her backpack in her car, so Dr. Greene lends her some paper and pens from his briefcase so she will be able to take notes in class. Dr. Greene drops off Annie and doesn't give his actions a second thought; after all, he was merely being helpful.

Slippery Slope

However, his actions set a precedent with Annie that a certain amount of nonprofessional interaction is acceptable.

Soon, their out-of-therapy relationship may involve socializing or dating, which would undoubtedly constitute an unethical circumstance in which Annie could eventually be exploited or harmed.

Although such “boundary erosion” is not inevitable, minor boundary infractions can foster the process.

As such, clinical psychologists should give careful thought to certain actions—receiving or giving gifts, sharing food or drink, self-disclosing their own thoughts and feelings, borrowing or lending objects, hugging—that may be expected and normal within most interpersonal relationships but may prove detrimental in the clinical relationship

Can the therapist and the client enter in an exclusive relationship after the therapy has been terminated?

Intimate Relationships

Most professional organizations prohibit their members from engaging in sexual relationships with former clients because of the potential for harm.

Some organizations specify a time period, and others do not.

Most of the organizations state that in the exceptional circumstance of sexual relationships with former clients—even after a 2- to 5-year interval—the burden of demonstrating that there has been no exploitation clearly rests with the therapist.

Some counselors maintain, “Once a client, always a client.”

APA Code of Ethics 10.08 b

Psychologists do not engage in sexual intimacies with former clients/patients after a two-year interval except in the most unusual circumstances

Psychologists who engage in such activity after the two years following the cessation or termination of therapy and of having no sexual contact with the former client bear the burden of demonstrating that there has been no exploitation, in light of relevant factors, including:

- The amount of time that has passed since therapy terminated
- The nature, duration and intensity of the therapy
- The circumstances of the termination
- Client's/patient's personal history
- Client's/patient's mental status
- The likelihood of adverse impact on the client/patient
- Any statements or actions made by the therapist made during therapy suggesting or inviting possibility of a post-termination sexual or romantic relationship with the client/patient

What can you say?

At this point, reflect on your own stance on the controversial issue of forming exclusive/intimate relationships once therapy has ended.

Should counselors be free to formulate their own practices about developing sexual relationships with former clients? Give your reasons.

Competence

- In general, competent clinical psychologists are those who are sufficiently capable, skilled, experienced, and expert to adequately complete the professional tasks they undertake (Nagy, 2012). One specific ethical standard in the section on competence (2.01a) addresses the boundaries of competence:
  - "Psychologists provide services, teach, and conduct research with populations and in areas only within the boundaries of their competence, based on their education, training, supervised experience, consultation, study, or professional experience" (American Psychological Association, 2002, p. 1063).
- ***An important implication of this standard is that having a doctoral degree or a license in psychology does not automatically make a psychologist competent for all professional activities***. Instead, the psychologist must be specifically competent for the task at hand
- This standard is consistent with the **continuing education regulations** of many state licensing boards. That is, to be eligible to renew their licenses, psychologists in many states **must attend lectures, participate in workshops, complete readings, or demonstrate in some other way that they are sharpening their professional skills and keeping their knowledge of the field current**.

Competence

- The APA's (2002) code of ethics also recognizes that **psychologists' own personal problems can lessen their competence**: "When psychologists become aware of personal problems that may interfere with their performing work-related duties adequately, they take appropriate measures, such as obtaining professional consultation or assistance, and determine whether they should limit, suspend, or terminate their work-related duties"
- On the professional side, the **phenomenon of burnout among clinical psychologists has been recognized in recent decades.** Burnout refers to a state of exhaustion that relates to engaging continually **in emotionally demanding work** that exceeds the normal stresses or psychological "wear and tear" of the job.
- As the ethical standard suggests, **psychologists should take action to prevent or minimize their own impairment**. Such actions can include **varying one's work responsibilities, keeping one's expectations reasonable, consulting with other professionals, maintaining a balanced and healthy personal life, or seeking psychotherapy, as necessary**.

Case Analysis

Dr. Aris is a clinical psychologist treating "Leo," a 24-year-old struggling with severe social anxiety and depressive episodes. One evening, while browsing a local community forum, Dr. Aris stumbles upon a post by Leo. In the post, Leo describes plans to quit his job and "disappear" because he feels his therapy isn't working and he feels "hopeless." He does not mention self-harm explicitly, but the tone is despondent.

In their next session, Leo acts normally and claims everything is "fine." Dr. Aris is unsure whether to bring up the forum post, as he wasn't "searching" for Leo, but knowing this information changes his clinical assessment of Leo’s risk level.

Criteria	Exceptional (5 pts)	Proficient (3-4 pts)	Developing (1-2 pts)	Score
Identification of Dilemma	Clearly identifies the conflicting ethical principles (e.g., Confidentiality vs. Duty to Warn).	Identifies the main issue but misses secondary ethical nuances.	Misidentifies the core ethical conflict.	/5
Application of Ethics Code	Correctly cites specific sections of the APA (or relevant) Ethics Code.	Mentions ethical concepts but lacks specific citations or rigor.	No reference to professional ethical codes.	/5
Clinical Reasoning	Demonstrates a deep understanding of how the ethics affect the therapeutic relationship.	Discusses clinical impact surface-level only.	Fails to consider the patient's psychological well-being.	/5
Cultural Competence	Evaluates how the patient’s background (race, age, SES) influences the ethical stakes.	Acknowledges culture but does not integrate it into the solution.	Ignores cultural or individual diversity factors.	/5
Proposed Resolution	Offers a practical, step-by-step plan that minimizes harm and maximizes benefit.	Resolution is logical but perhaps idealistic or slightly impulsive.	Resolution is unethical or practically impossible.	/5
TOTAL SCORE			**Grand Total:**	**/30**

Diagnostic and Classification Issues: DSM5 and More

Chapter VII

Outline

Defining normality and abnormality

What defines abnormality?

Who defines abnormality

Diagnostic and Classification of Mental Disorders: A Brief History

Defining Normality and Abnormality

What defines abnormality?

- These answers have included criteria such as **personal distress to the individual** (as in severe depression or panic disorder), **deviance from cultural norms** (as in many cases of schizophrenia), **statistical infrequency** (as in rarer disorders such as dissociative identity disorder), and **impaired social functioning** (as in social phobia and, in a more dangerous way, antisocial personality disorder).
- Wakefield (1992) explains his harmful dysfunction: ***"I argue that a disorder is a harmful dysfunction, wherein harmful is a value term based on social norms, and dysfunction is a scientific term referring to the failure of a mental mechanism to perform a natural function for which it was designed by evolution. Thus, the concept of disorder combines value and scientific components."***
- The harmful dysfunction theory proposes that in our efforts to determine what is abnormal, we consider both scientific (e.g., evolutionary) data and the social values in the context of which the behavior takes place.

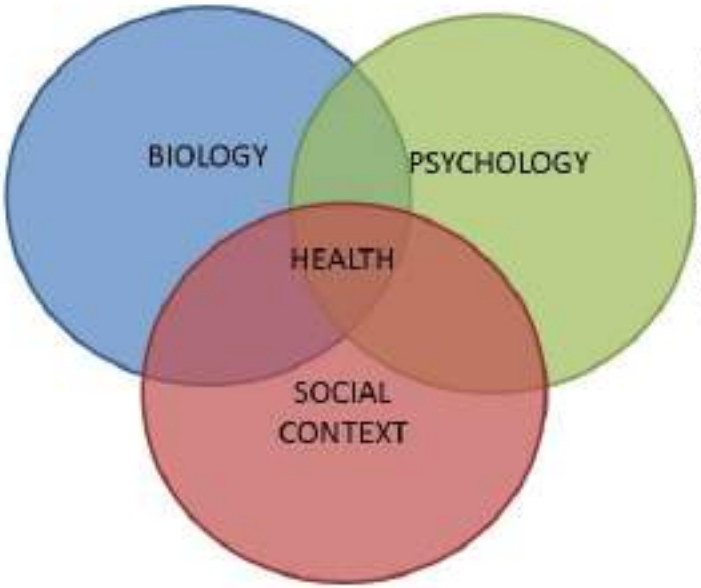
Who defines abnormality?

- We use disorders—as defined by the Diagnostic and Statistical Manual of Mental Disorders—every day as they perform assessments, conduct therapy, and design and execute research studies.
- In DSM-5, mental disorder is defined as a **"clinically significant disturbance" in "cognition, emotion regulation, or behavior" that indicates a "dysfunction" in "mental functioning" that is "usually associated with significant distress or disability" in work, relationships, or other areas of functioning.**
- In addition*, the definition states that expectable reactions to common stressors are not mental disorders.*
- Who created this definition, as well as the specific diagnostic categories that fill DSM?
    - This group consisted largely of leading researchers in various specialty areas within **psychopathology** who were selected for their scholarship and expertise in their respective fields. It is noteworthy that this task force consisted primarily of **psychiatrists**, and a relatively small number of **psychologists** and **other mental health professionals** were included.

What defines abnormality?

4 Ds of Diagnosis	
Deviance	• Behaviors, thoughts, emotions • Rare within the society, different from norms
Dysfunction	• One's ability to continue carrying daily activity • Maladaptive behavior
Distress	• Unpleasant and upsetting • Display of great deal of anxiety, depression, sadness, etc.
Danger	• To the individual himself/herself • To the people around him/her

BIOPSYCHOSOCIAL APPROACH TO UNDERSTANDING HEALTH



Gender
Physical illness
Disability
Genetic vulnerability
Immune function
Neurochemistry
Stress reactivity
Medication effects

Learning/memory
Attitudes/beliefs
Personality
Behaviours
Emotions
Coping skills
Past trauma

Social supports
Family background
Cultural traditions
Social/economic status
Education

Why is the definition of abnormality important?

- As an example, consider attenuated psychosis syndrome. Currently, attenuated psychosis syndrome is not an official diagnostic category. Instead, it is listed as a proposed criteria set in the "Emerging Measures and Models" section of DSM-5. This section describes conditions that DSM authors decided to leave **out of the list of "official" disorders, at least for now, but to list as "unofficial" conditions for the purpose of inspiring clinicians and researchers to study them more.**
- Importance for professionals
  - DSM authors have facilitated the study of attenuated psychosis syndrome by researchers and its consideration by clinicians. If attenuated psychosis syndrome becomes an official diagnosis, **we will undoubtedly see an increase in both these activities**. Additionally, if attenuated psychosis syndrome becomes official, **people will be diagnosed with it and will be conceptualized (by professionals and themselves) as having this form of mental illness**.
- Importance for clients
  - Future decisions by DSM authors about the status of attenuated psychosis syndrome may carry special significance. Some clients could experience **beneficial consequences of being diagnosed with attenuated psychosis syndrome**
  - the decisions DSM authors make when defining abnormality, both as a broad concept and as specific diagnostic categories, **profoundly influence many aspects of clients' lives.**

Diagnostic and Classification of Mental Disorders: A Brief History

Before DSM

- **Hippocrates** (460–377 BCE) wrote extensively about abnormality, but unlike most of his predecessors, he did not offer supernatural explanations such as possession by demons or gods. Instead, **his theories of abnormality emphasized natural causes**.
- the 19th century, we find an era when many cities in Europe and the United States were establishing **asylums** for the treatment of the mentally ill (as opposed to the imprisonment or abuse they might have suffered previously).
- Around 1900, more important steps were taken toward the eventual DSM system that we currently use
- During the late 1800s and early 1900s, the primary purpose of diagnostic categories was the collection of **statistical and census data**. Later, **in the mid-1900s, the U.S. Army and Veterans Administration (now Veterans Affairs) developed their own early categorization system to facilitate the diagnosis and treatment of soldiers returning from World War II**

DSM: Earlier Editions (I and II)

- **DSM-I was published by the American Psychiatric Association in 1952. DSM-II followed as a revision in 1968**. These two editions of the DSM were quite similar to each other, but as a pair, they were quite different from all the DSM editions subsequently published.
- DSM-I and DSM-II contained only three broad categories of disorders:
  - **psychoses** (which would contain today's schizophrenia),
  - **neuroses** (which would contain today's major depression, bipolar disorder, and anxiety disorders), and
  - **character disorders** (which would contain today's personality disorders)
- **The definitions of disorders in DSM-I and DSM-II were not scientifically or empirically based.** Instead, they represented "the accumulated clinical wisdom of the small number of senior academic psychiatrists who staffed the DSM task forces"
- Most of these psychiatrists **were psychoanalytic in orientation**, and the language of the first two DSM editions reflected the psychoanalytic approach to understanding people and their problems.
- The descriptions of individual disorders in DSM-I and DSM-II were not lists of specific symptoms or criteria; instead, they **were simply prose, typically one paragraph per disorder, offering relatively vague descriptions of clinical conditions**. As a result, the diagnostic categories in DSM-I and DSM-II had very limited generalizability or utility for clinicians in practice at the time

000–x61 Antisocial reaction

This term refers to chronically antisocial individuals who are always in trouble, profiting neither from experience nor punishment, and maintaining no real loyalties to any person, group, or code. They are frequently callous and hedonistic, showing marked emotional immaturity, with lack of sense of responsibility, lack of judgment, and an ability to rationalize their behavior so that it appears warranted, reasonable, and justified.

The term includes cases previously classified as "constitutional psychopathic state" and "psychopathic personality." As defined here the term is more limited, as well as more specific in its application.

000–x62 Dyssocial reaction

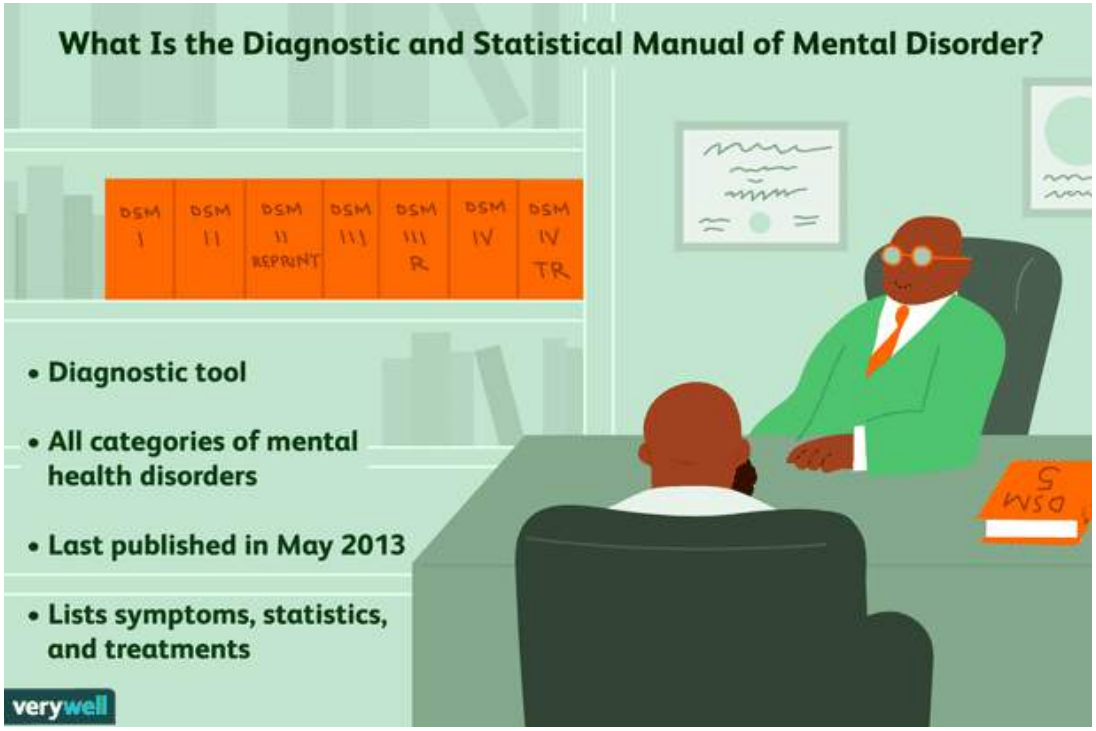
This term applies to individuals who manifest disregard for the usual social codes, and often come in conflict with them, as the result of having lived all their lives in an abnormal moral environment. They may be capable of strong loyalties. These individuals typically do not show significant personality deviations other than those implied by adherence to the values or code of their own predatory, criminal, or other social group. The term includes such diagnoses as "pseudosocial personality" and "psychopathic personality with asocial and amoral trends."

DSM—More Recent Editions (III, III-R, IV, and IV-TR)

- In comparison to DSM-I and DSM-II, it reflected an approach to defining mental disorders that differed substantially in some important ways:
  - It relied to a **much greater extent on empirical data** to determine which disorders to include and how to define them.
  - It used specific **diagnostic criteria to define disorders**. Whereas the DSM-III retained some descriptive paragraphs (and in fact augmented them for most disorders), these paragraphs were followed by specific criteria—checklists, basically—that delineated in much greater detail the symptoms that must be present for an individual to qualify for a diagnosis.
  - It **dropped any allegiance to a particular theory of therapy or psychopathology**. As a result, the psychoanalytic language of previous editions was replaced by terminology that reflected no single school of thought.
- It introduced the **multiaxial assessment system** that remained in DSM through the next several editions but was dropped in DSM-5.
- DSM-III offered extended descriptions and added lists of specific criteria. In addition, it included many new disorders—265 disorders in total, as compared with 182 in DSMII and 106 in DSM-I.

DSM-5

- In May 2013, DSM-5 was published. It was the first substantial revision of the manual in about 20 years, and it was a massive effort. Led by two prominent mental health researchers, David Kupfer and Darrel Regier.
- Many steps were involved in this huge undertaking. Early on, a **Task Force**—essentially, a committee of prominent researchers in various areas of mental disorders—was created. The members of this Task Force then led Work Groups, each of which focused on a particular area of mental disorders (e.g., the Eating Disorders Work Group, the Psychotic Disorders Work Group).
- These Work Groups **reviewed the disorders listed in the previous DSM and considered proposals for revision, including ideas for adding, eliminating, combining, splitting, or revising the definitions of disorders.'**
- The leaders also created a **Scientific Review Committee** of experts separate from the Work Groups whose job was **to make sure that there was sufficient scientific evidence to support the changes proposed by the Work Groups.**
- Throughout the process, the DSM-5 authors tried to coordinate their efforts with those of the World Health Organization (WHO), which publishes the International Classification of Diseases (ICD).



Our changing conception of autism / ASD

DSM-3 (1980)

Diagnostic and Statistical Manual of Mental Disorders

Mental Disorders

Neurodevelopmental Disorders

Pervasive Development Disorders

Infantile Autism
- Onset before 30 months
- Lack of responsiveness to other people
- Gross deficits in language development
- Resistance to change
- Attachment to inanimate objects

Childhood Onset PDD
- Onset after 30 months
- Gross impairment in social relationships
- Anxiety and panic attacks
- Inappropriate fear or rage reactions
- Resistance to change
- Oddities of motor movement and speech
- Hyper-sensitive to sensory stimuli

Atypical Autism
Distortions in the development of multiple basic psychological functions that are involved in the development of social skills and language and that cannot be classified as either Infantile Autism or Childhood Onset PDD

In DSM-1 (1952) and DSM-2 (1968), autism was not listed as a disorder. Autistic behaviour was mentioned only as part of the description of childhood schizophrenia.

VaccineEvidence.com

DSM-4 (1994)

Pervasive Development Disorders

1. Impairments in social interaction
2. Impairments in communication
3. Restricted, repetitive and stereotyped patterns of behavior, interests and activities

Autistic Disorder
- Onset before 36 months
- Two or more impairments in social interaction
- One or more impairments in communication
- One or more manifestations of restricted behaviours
- A total of 6 or more impairments

PDD-NOS (Not Otherwise Specified)
Severe and pervasive impairments in at least one of the three areas of diagnosis for Autistic Disorder, but the criteria are not met for any of the specific PDDs

Asperger's Disorder
- Two or more impairments in social interaction
- No impairments in communication
- One or more manifestations of restricted behaviours
- No delay in cognitive or language development

Childhood Disintegrative Disorder
- Regression in multiple areas of functioning after at least 2 years of apparently normal development.
- Clinically significant loss of previously acquired skills such as language, social skills, bowel or bladder control, play and motor skills and cognitive abilities.
- Impairments after regression in at least two of the three areas of diagnosis for Autistic Disorder.

Rett Disorder
- Onset of deceleration of head growth between 5 and 48 months
- Loss of previously acquired hand skills, social engagement and language skills
- Poorly coordinated gait and trunk movements.

DSM-5 (2013)

Autism Spectrum Disorders (ASD)

A. Persistent deficits in social communication and social interaction
B. Restricted, repetitive patterns of behavior, interests, or activities

	Social Deficits	Restricted Behaviors
Severe (Level 3) Requiring very substantial support	- Severe deficits in verbal and nonverbal social communication - Severe impairments in functioning - Very limited initiation of social interactions - Minimal response to social overtures from others	- Inflexibility of behavior - Extreme difficulty coping with change - Restricted/repetitive behaviors markedly interfere with functioning in all spheres. - Great distress/difficulty changing focus or action.
Moderate (Level 2) Requiring substantial support	- Marked deficits in verbal and nonverbal social communication - Social impairments apparent even with supports in place - Limited initiation of social interactions - Reduced or abnormal responses to social overtures from others.	- Inflexibility of behavior - Difficulty coping with change - Restricted/repetitive behaviors appear frequently enough to be obvious to the casual observer and interfere with functioning in a variety of contexts. - Distress and/or difficulty changing focus or action.
Mild (Level 1) Requiring support	- Without supports in place, deficits in social communication cause noticeable impairments. - Difficulty initiating social interactions - Clear examples of atypical or unsuccessful response to social overtures of others. - May appear to have decreased interest in social interactions.	- Inflexibility of behavior causes significant interference with functioning in one or more contexts. - Difficulty switching between activities. - Problems of organization and planning hamper independence.

Social (Pragmatic) Communication Disorder
Meets the social deficits criteria of ASD, but not the restricted behaviours criteria

DSM-II (American Psychiatric Association, 1968): 300.0 Anxiety Neurosis	*DSM-5* (American Psychiatric Association, 2013): 300.02 Generalized Anxiety Disorder
This neurosis is characterized by anxious over-concern extending to panic and frequently association with somatic symptoms. Unlike Phobic neurosis, anxiety may occur under any circumstances and is not restricted to specific situations or objects. This disorder must be distinguished from normal apprehension or fear, which occurs in realistically dangerous situations.	Excessive anxiety and worry (apprehensive expectation), occurring more days than not for at least 6 months, about a number of events or activities (such as work or school performance). The person finds it difficult to control the worry. The anxiety and worry are associated with three (or more) of the following six symptoms (with at least some symptoms present for more days than not for the past 6 months). Note: Only one item is required in children. 1. restlessness or feeling keyed up or on edge 2. being easily fatigued 3. difficulty concentrating or mind going blank 4. irritability 5. muscle tension 6. sleep disturbance (difficulty falling or staying asleep, or restless unsatisfying sleep) The anxiety, worry, or physical symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning. The disturbance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism). The disturbance is not better explained by another mental disorder (e.g., anxiety or worry about having panic attacks in panic disorder, negative evaluation in social anxiety disorder [social phobia], contamination or other obsessions in obsessive-compulsive disorder, separation from attachment figures in separation anxiety disorder, reminders of traumatic events in posttraumatic stress disorder, gaining weight in anorexia nervosa, physical complaints in somatic symptom disorder, perceived appearance flaws in body dysmorphic disorder, having a serious illness in illness anxiety disorder, or the content of delusional beliefs in schizophrenia or delusional disorder).

Version	Year	Number of diagnoses
DSM-I	1952	102
DSM-II	1968	182
DSM-III	1980	265
DSM-III-R	1987	292
DSM-IV	1994	297
DSM-IV-TR	2000	365
DSM-5	2013	157

DSM-5-TR

is the most comprehensive, current, and critical resource for clinical practice available to today's mental health clinicians and researchers.

With contributions from over 200 subject matter experts, this fully updated volume boasts the most current text updates based on the current literature. Now better than ever, *DSM-5-TR* provides a cohesive, updated presentation of criteria, diagnostic codes, and text.

Some changes

Used in DSM-5

- Desired gender
- Cross-sex medical procedure
- Natal male
- Natal female

Used in DSM-5-TR

- Experienced gender
- Gender affirming medical procedure
- Individual assigned male at birth
- Individual assigned female at birth

- The parenthetical "(social phobia)" next to social anxiety disorder was removed. The term "intellectual disability" was revised to intellectual development disorder.
- The DSM-5-TR also made significant revisions to terms surrounding gender dysphoria (Cherry, 2022).

The DSM-5-TR also made changes aimed at reducing racial and cultural biases. These revisions include:

- The term "race" was replaced with "racialized" to call out that race is socially constructed.
- The term "ethnoracial" is used to refer to categories like Hispanic, White, and African American.
- The terms "minority" and "non-White" are not used because they imply that Whiteness is prioritized over other social groups.
- The term "Caucasian" is not used. The APA notes that this term is based on erroneous views about the geographic origin of people who are called Caucasian.
- The term Latinx is used instead of Latino/Latina for gender inclusivity.

To love is to will the good of the other.

-St. Thomas Aquinas

The Clinical Interview

Chapter VIII

The Clinical Interview

- An element common to all kinds of psychological assessment is **feedback**. In other words, clinical psychologists **provide their clients with the results of tests or interviews that have been conducted**.
- This feedback can come in the form of **a face-to-face meeting, a written report, or other forms**. Psychologists learn to provide feedback during graduate training, but postgraduate internships and professional experience also shape the way they provide feedback to clients
- **Most psychologists believe that clients find their feedback to be helpful and positive, even before any type of intervention (e.g., psychotherapy) is implemented.**

The Interviewer

The most pivotal element of a clinical interview is the person who conducts it.

A skilled interviewer only is not a master of the technical and practical aspects of the interview but also demonstrates broad-based wisdom about the human interaction it entails.

General Skills

Quieting yourself

what should be quieted is the interviewer's **internal**, self-directed thinking pattern. To the extent that interviewers are preoccupied by their own thoughts, they will struggle in the fundamental task of listening to their clients.

The voice in the interviewer's own mind should not interrupt or drown out the voice of the client.

Being self-aware

It should not be confused with the excessive self-consciousness. The type of self-awareness that should be maximized is the **interviewer's ability to know how he or she tends to affect others interpersonally** and how others tend to relate to him or her.

Skilled interviewers **are cognizant of their unique traits, including their cultural assumptions as well as more superficial characteristics, and consider their effect on the interview process**

Developing positive working relationships

There is no formula for developing positive working relationships during an interview; however**, attentive listening, appropriate empathy, genuine respect, and cultural sensitivity play significant roles.** Positive working relationships are always a function of the interviewer's attitude as well as the interviewer's actions

Specific Behaviors

Eye Contact

- Eye contact not only facilitates listening, but it also **communicates listening**. That is, when the client notices the interviewer's continuous, appropriate eye contact, the client feels heard.
- Like so many other aspects of the clinical interview, *eye contact is a specific behavior that requires cultural knowledge and sensitivity on the part of the interviewer*, both as the sender and receiver of eye contact.

Body Language

- There are a few general rules for the interviewer: **face the client, appear attentive, minimize restlessness, display appropriate facial expressions, and so on.**
- Furthermore, the client's body language can be misinterpreted by an interviewer whose knowledge of the client's cultural background is deficient.

Vocal Qualities

- Skilled interviewers have mastered the subtleties of the vocal qualities of language—not just the words but how those words sound to the client's ears. They **use pitch, tone, volume, and fluctuation in their own voices to let clients know that their words and feelings are deeply appreciated**. And these interviewers also attend closely to the vocal qualities of their clients.

Specific Behaviors

Tracking

- Verbal Tracking
  - Effective interviewers are able to **repeat key words and phrases** back to their clients to assure the clients that they have been accurately heard.
  - In addition, interviewers skilled at verbal tracking **monitor the train of thought implied by clients' patterns of statements** and are thus able to shift topics smoothly rather than abruptly.

Referring

- Referring to the Client by their Proper Name
  - Inappropriately **using nicknames** or shortening names (e.g., calling Benjamin "Benji"), omitting a "middle" name that is in fact an essential part of the first name (e.g., calling John Paul "John"), or addressing a client by first name rather than a title followed by surname (e.g., addressing Ms. Washington as "Latrice") are presumptuous mistakes that can jeopardize the client's sense of comfort with the interviewer

Specific Behavior

- Observing Client Behavior
  - ***All of the previously described interviewer behaviors, and many other important decisions the interviewer makes, can be informed by behavioral observations of the client by the interviewer***.
  - Typically, when psychologists write a report summarizing the results of an assessment (including the clinical interview), ***that report contains at least a brief section describing the behavior of the client during the process***.
  - That behavior can vary widely: Some clients are calm while others are nervous; some are easygoing while others are hostile; some stay on task while others stray in random directions; some are emotional while others are stoic; and so on.
  - Observing client behaviors during the interview **allows the psychologist to consider not only what the client said but how the client said it, and that "how" component can offer important information to the psychologist about responding during the interview and understanding the client.**

Components of the Interview

Rapport

Rapport refers to a positive, comfortable relationship between interviewer and client. They feel that the interviewers have “connected” with them and that the interviewers empathize with their issues

Rapport

First, interviewers should try to put the client at ease. For some clients, the first visit to a clinical psychologist can be daunting. They may be fearful about the interview or may be under a misperception of what the session will entail. **Engaging in small talk about innocuous topics.**

Second, interviewers can acknowledge the unique, unusual situation of the clinical interview. From the client's perspective, **the interview experience is unlike any other interaction**: *They are expected to discuss private, difficult, often emotionally charged issues with a person they just met and know very little about. Letting clients know that you recognize their position and appreciate their willingness to participate communicates empathy and enhances rapport*. **Along the same lines, inviting clients to ask questions about the interview process provides them with a sense of knowledge and control, which can also improve their comfort level**

Third, interviewers can enhance rapport **by noticing how the client uses language and then following the client's lead.** Interviewers should pick up on the client's vocabulary and, as much as possible, speak in similar terms

Technique

If rapport is how an interviewer is with clients, technique is what an interviewer does with clients.

These are the tools in the interviewer’s toolbox, including questions, responses, and other specific actions.

Directive vs Nondirective Styles

Directive questions tend to be targeted toward specific pieces of information, and client responses are typically brief, sometimes as short as a single word (e.g.,“yes” or “no”).

- Direct questioning can provide crucial data that clients may not otherwise choose to discuss: important **historical information** (“How often have you attempted suicide in the past?” “Is there a history of schizophrenia in your family?”), **the presence or absence of a particular symptom of a disorder** (“How many hours per day do you typically sleep?”), **frequency of behaviors** (“How often have you had panic attacks?”), and **duration of a problem** (“How long has your son had problems with hyperactivity?”), among other information

On the other hand, interviewers who use a nondirective style allow the client to determine the course of the interview. Without direction from the interviewer, a client may choose to spend a lot of time on some topics and none on others.

- Indirect questioning, conversely, can provide crucial information that interviewers may not otherwise know to inquire about.
- “Can you tell me more about your feelings of sadness?”
- indirect question allows to **expand on anything he believes is essential, including symptoms, duration, background data, or other elements that may prove to be extremely relevant**

Specific Interviewer Responses

- Ultimately, interviewing technique consists of what the interviewer chooses to say. The interviewer's questions and comments can span a wide range and serve many purposes.
- There are numerous common categories of interviewer responses: open- and closed-ended questions, clarification, confrontation, paraphrasing, reflection of feeling, and summarizing

Open- and Close-Ended Questions

- An open-ended question, such as, "**Can you please tell me more about the eating problems you mentioned on the phone?"** may take the interview in a different direction than a closed-ended question, such as, "How many times per week do you binge and purge?" "Which purging methods do you use—vomiting, exercise, nausea?" or "Have you been diagnosed with an eating disorder in the past?"
- **Open-ended questions allow for individualized and spontaneous responses from clients**. These responses tend to be relatively long, and although they may include a lot of information relevant to the client, they may lack details that are important to the clinical psychologist

Open- and Close-Ended Questions

Closed-ended questions allow for far less elaboration and self-expression by the client but yield quick and precise answers.

Indeed, open-ended questions are the building blocks of the nondirective interviewing style described above, whereas the directive interviewing style typically consists of closed-ended questions.

Clarification

The purpose of a clarification question is **to make sure the interviewer has an accurate understanding of the client's comments.** Clarification questions not only enhance the interviewer's ability to "get it," *they also communicate to the client that the interviewer is actively listening and processing what the client says*.

For example, with Brianna (the client with bulimic symptoms described above), the interviewer may at one point say, "You mentioned that a few months ago you started exercising excessively after eating large amounts of food but that you've never made yourself vomit—do I have that right?" or "I want to make sure I'm understanding this correctly—did you mention that you've been struggling with eating-related issues for about 6 months?"

Either of these questions would allow Brianna to confirm or correct the interviewer so that ultimately the client is correctly understood.

Confrontation

Interviewers use confrontation w**hen they notice discrepancies or inconsistencies in a client's comments**. Confrontations can be like clarifications, but **they focus on apparently contradictory information** provided by clients.

For example, an interviewer might say to Brianna, "Earlier, you mentioned that you had been happy with your body and weight as a teenager, but then a few minutes ago you mentioned that during high school you felt fat in comparison with many of your friends. I'm a bit confused."

Paraphrasing

Unlike clarifying or confronting, paraphrasing is not prompted by the interviewer's need to resolve or clarify what a client has said. Instead, paraphrasing is **used simply to assure clients that they are being accurately heard.**

When interviewers paraphrase, they typically restate the content of clients' comments, using similar language.

So if Brianna mentions, "I only binge when I'm alone," the interviewer might immediately respond with a statement such as, "You only binge when no one else is around."

A paraphrase usually doesn't break new ground; instead, **it maintains the conversation by assuring the client that the interviewer is paying attention and comprehending.**

Reflection of feeling

Reflections of feeling are intended to make clients feel that their emotions are recognized, even if their comments did not explicitly include labels of their feelings.

For example, if Brianna's comment above ("I only binge when I'm alone") was delivered with a *tone and body language that communicated shame*—her hand covering her face, her voice quivering, and her eyes looking downward—the interviewer might respond with a statement like, **"You don't want anyone to see you binging—do you feel embarrassed about it?"**

Unlike paraphrasing, reflecting a client's feelings often involves an **inference by the interviewer about the emotions underlying the client's words**.

Summarizing

- Summarizing usually involves **tying together various topics that may have been discussed, connecting statements that may have been made at different points, and identifying themes that have recurred during the interview.**
- Like many of the other responses described above, summarizing **lets clients know that they have been understood but in a more comprehensive, integrative way** than, say, paraphrasing single statements.
- After a full interview with Brianna, an interviewer may summarize by offering statements along the lines of**, "It seems as though you are acknowledging that your binging and purging have become significant problems in recent months, and while you've kept it to yourself and you may feel ashamed about it, you're willing to discuss it here with me and you want to work toward improving it."**
- An accurate summarization conveys to the client that the interviewer has a good grasp on the **"big picture."**

Conclusions

The conclusion can take a number of different forms, depending on the type of interview, the client's problem, the setting, or other factors.

In some cases, the conclusion can be essentially **similar to a summarization**, as described above.

Or the interviewer might be able to go a step further by **providing an initial conceptualization of the client's problem** that incorporates a greater degree of detail than a brief summarization statement.

In some situations, the conclusion of the interview may consist of a **specific diagnosis** made by the interviewer based on the client's response to questions about specific criteria.

Or the conclusion may involve **recommendations**. **These recommendations might include outpatient or inpatient treatment, further evaluation** (by another psychologist, psychiatrist, or health professional), or any number of other options.

Pragmatics of the Interview

Note Taking

- There are good reasons for taking notes. Written notes are certainly more reliable than the interviewer's memory. Many clients will expect the interviewer to take notes and may feel as though their words will soon be forgotten if the interviewer is not taking notes.
- On the other hand, there are also drawbacks to taking notes. The process of note taking can be a **distraction**, both for the interviewer, who may fail to notice important client behaviors while looking down to write, and for the client, who may feel that the interviewer's notebook is an obstacle to rapport.
- **It is likely that the effect of note taking is highly dependent on the situation**, including exactly how a client interprets the use of any mode of note taking and exactly how an interviewer uses that mode.

Audio and Video Recording

Clinical psychologists may prefer to audio or video-record the session. Unlike note taking, recording a client's interview requires that the interviewer obtain written permission from the client.

While obviously providing a full record of the entire session, recordings can, with some clients, hinder openness and willingness to disclose information.

As with note taking, an explanation of the rationale for the recording, as well as its intended use (e.g., review by the interviewer or a supervisor) and a date by which it will be erased or destroyed, is typically appreciated by the client.

Interview Room

As a general rule, **“when choosing a room [for interviews], it is useful to strike a balance between professional formality and casual comfort”.**

That is, the interview room should subtly **convey the message to the client that the clinical interview is a professional activity but one in which warmth and comfort are high priorities.**

There are quite a few options: **the traditional (and increasingly rare) psychoanalytic arrangement in which the client lies on a couch while the interviewer sits in a chair**, **interviewer and client sitting face-to-face, interviewer and client in chairs at an angle between 90° and 180°, and others**.

Interview Room

- A table or desk may be positioned between the chairs, small side tables may accompany each chair, or the chairs may have no furniture between them at all. And depending on the setting, the chairs themselves may run the gamut of anything available at office- or home-furniture stores.
- **As long as the interview room facilitates the fundamental goals of the interview, such as gathering information and building rapport in a private setting free of interruptions, its purpose is well served**
- **When making decisions related to the decor of an office, clinical psychologists usually steer clear of overtly personal items such as family photos, souvenirs, and memorabilia**. Such items might be inconsistent with the professional ambience that the interview room should exude, and, perhaps more important, they can influence the content of the interview in some cases.

Confidentiality

- To inform their clients about confidentiality interviewers should routinely explain policies regarding confidentiality as early as possible. Such explanations should be consistent with state law and professional ethics and provided in writing, with ample opportunity for oral discussion offered as well.
- Psychologists who discuss confidentiality and its limits with interview clients demonstrate competent and ethical practice

Types of Interview

Intake Interviews

The purpose of the intake interview is essentially to determine whether to “intake” the client to the setting where the interview is taking place.

In other words, **the intake interview determines whether the client needs treatment**; if so, what form of treatment is needed (inpatient, outpatient, specialized provider, etc.); **and whether the current facility can provide that treatment, or the client should be referred to a more suitable facility.**

Intake interviews typically involve detailed questioning about the presenting complaint.

Example

Consider Julia, a 45-year-old client who arrives for an intake interview at an outpatient community mental health center and describes "hearing voices." Dr. Epps, the clinical psychologist conducting the intake interview, is likely to ask questions about the frequency, intensity, and duration of Julia's psychotic symptoms, as well as symptoms of other conditions that Julia may exhibit. Dr. Epps would also closely observe Julia during the interview and note any relevant behaviors. Questions about Julia's personal history, especially the previous existence of this or other psychological or psychiatric problems, would also be an important part of the intake interview. At the conclusion of the interview, Dr. Epps would use all information solicited to decide whether Julia's problems would be best treated at this community mental health center and, if so, recommend a particular mode or frequency of treatment. If not, Dr. Epps would provide Julia with referrals to other treatment providers, including inpatient or outpatient settings as appropriate.

Diagnostic Interviews

As the name indicates, the purpose of the diagnostic interview is to diagnose. At the end of a well-conducted diagnostic interview, **the interviewer is able to confidently and accurately assign Diagnostic and Statistical Manual of Mental Disorders (DSM) diagnoses to the client's problems.**

When an interview yields a valid, specific diagnosis, **the effectiveness of the recommendations and subsequent treatment may be increased**. **If the purpose of the diagnostic interview is to produce a diagnosis, it would make sense for the diagnostic interview to include questions that relate to the criteria of DSM disorders.**

How directly should the questions reflect the diagnostic criteria? Some clinical psychologists believe that the questions should essentially replicate the DSM criteria; after all, if our profession follows a diagnostic manual with disorders defined by meticulous lists of symptoms, what better way to reach a diagnosis than to ask directly about each symptom?

Other clinical psychologists believe that every question in a diagnostic interview need not be coupled with a specific DSM diagnostic criterion. These clinical psychologists prefer a more flexible interview style in which they choose or create questions as they move through the interview, and they use inference rather than absolute fact to make diagnostic decisions.

In other words, some clinical psychologists prefer structured interviews, while others prefer unstructured interviews.

Structured Interviews vs Unstructured Interviews

A structured interview is a predetermined, planned sequence of questions that an interviewer asks a client. Structured interviews are constructed for purposes, usually diagnostic.

An unstructured interview, in contrast, involves no predetermined or planned questions. In unstructured interviews, interviewers improvise: They determine their questions on the spot, seeking information that they decide is relevant during the interview

Mental Status Exam

The mental status exam is employed most often in medical settings. Its primary purpose is to quickly assess how the client is functioning at the time of the evaluation.

Instead, its yield is usually a brief paragraph that captures the psychological and cognitive processes of an individual “right now”—like a psychological snapshot

The format of the mental status exam is not completely standardized, so it may be administered differently by various health professionals. (Psychiatrists, in addition to clinical psychologists and other mental health professionals, often use the mental status exam.)

Mental Status Exam

- Appearance
- Behavior/psychomotor activity
- Attitude toward examiner
- Affect and mood
- Speech and thought
- Perceptual disturbances
- Orientation to person, place, and time
- Memory and intelligence
- Reliability, judgment, and insight

Things to consider

Level of consciousness

This refers to the client's level of alertness and responsiveness to questions or other stimuli.

Orientation

Awareness of date/time, current location, and current situation (e.g., reason for appointment).

Gross/fine motor movement

The client's gait, posture, manual dexterity, etc.

Dress/grooming and hygiene

Is the client neatly dressed or more disheveled? Neatly or poorly groomed? Are they attending to personal hygiene?

Things to consider

Speech	A client's speech pattern is typically described in terms of fluency, rate, volume, and intonation, as well as any odd tonality, such as in foreign accent syndrome. it carries much verbal and nonverbal information, including about emotional state and coherence of thought.
Affect	A client's range of emotional expression, based on their speech, facial expressions, or other behaviors.
Mood	What the client reports about their internal mood state, especially as concerns any depressive or anxious symptoms.
Attention/working memory	Ability to focus on tasks (attention) and briefly hold information in mind (working memory) before using it. This domain is based on examiner observations, client self-report, or brief tests such as counting backward from 100 by 7s.
Memory	Ability to recall information, based on examiner observations, client self-report, or brief tests, such as short-term recall of three objects stated to them.

Crisis Interviews

- They are designed not only to assess a problem demanding urgent attention (most often, clients actively considering suicide or another act of harm toward self or others) but also **to provide immediate and effective intervention for that problem**
- Can be conducted in person but also take place often on the telephone via suicide hotlines, crisis lines, and similar services.
- Quickly establishing rapport and expressing empathy for a client in crisis, especially a suicidal client, are key components of this type of interview.
- Providing an immediate, legitimate alternative to suicide can enable the client to endure this period of very high distress and reach a later point in time when problems may feel less severe or solutions may be more viable

Communication Drills

Traumatology Institute and Psychological Services

Dr. Anna B. Baranowsky, Ph.D., C.Psych.,

Executive Director & Dr. J. Eric Gentry, Ph.D., Consulting Director

Michael Rank, PhD, & Kathy Regan Figley, MS, MT

Communication Drills

Teresa Descillo was the developer of the following training drills

Each drill helps to enhance various aspects of effective interpersonal communication with trauma survivors (e.g. reflective listening, mirroring, paraphrasing, questioning, non-imposing or judgmental interacting).

There are four types of drills developed for this exercise:

- Focusing Drill 1, 2, & 3
- Attitude Adjustment Drill
- Acknowledgment Drill
- Closure Drill

Focus Drill #1

- In this drill you will take the “drill position” which is face-to-face sitting in front of your partner close enough so your knees almost touch. The object is to hold with position remaining aware of the other person, with our eyes closed and our body still - feet flat on floor and hands on lap. We are to remain aware of our partner as opposed to the wandering messages in our mind or physical aspects of our own bodies.

Focus Drill #2

- Holding the same position as above open your eyes and focus on your partner. The goal is to remain attentive and still giving your attention to the other person while releasing personal internal dialogue and physical discomfort. In order for us to work effectively with trauma survivors we need to establish a solid ability to be with others despite any internal discomfort we may be experiencing. With trauma work, as many responders/clinicians can attest to this is mostly about our own terror of encountering trauma in our own lives or the triggering of emotions with reflecting upon another’s loss, grief or trauma story.

- **Focusing Drill #3**
  - Again we maintain the “drill position”. This time the dyad has two roles the “learner” and the “trainer”. It is the trainer’s responsibility to run the drill and teach the “learners” the drill objectives.
  - Objective 1
    - “Begin a drill by telling your partner to start. If the student reacts or needs correction for an error, give a time-out signal, saying time out, and tell the student what the mistake was. And then tell
  - Objective 2
    - Repeat whatever caused the reaction. Or, in later drills, have the student repeat a phrase [or action] that caused the reaction (Descillo, 1997, p. 9).”
- The goal of this drill is to reduce “reactivity” and enhance ability to remain “intentional”. This will allow us to decide that we are prepared to do trauma work and truly listen and then be able to follow through as we are no longer susceptible to random “triggers”.